

MEDICAL CONSULTATION REPORT

Date of Report: February 4, 2026 Facility: City Central General Hospital

Department: Internal Medicine

1. PATIENT INFORMATION

Name: Jonathan Miller

Age: 35 Years

Gender: Male

Patient ID: #CM-2026-0982

Date of Consultation: February 4, 2026

2. CLINICAL PRESENTATION

Chief Complaint: Patient presents with a sudden onset of elevated blood pressure readings.

History of Present Illness: The patient is a 35-year-old male who reports a recent spike in blood pressure. The most recent measurement was recorded at 140/90 mmHg. He has no prior history of hypertension and is currently asymptomatic, denying headaches, dizziness, chest pain, or visual disturbances.

Medical History:

Chronic Conditions: None.

Surgical History: None.

Medications: None.

Allergies: No Known Drug Allergies (NKDA).

Family History:

Significant for hypertension in the father and maternal grandfather.

Social History:

Non-smoker; occasional alcohol use.

Reports high work-related stress.

Sedentary lifestyle with a diet high in processed, high-sodium foods.

3. PHYSICAL EXAMINATION & VITALS

Blood Pressure: 140/90 mmHg

Heart Rate: 78 bpm

Respiratory Rate: 16 breaths/min

Temperature: 98.4°F

General Appearance: Well-nourished, alert, and in no acute distress.

Cardiovascular: Regular rate and rhythm; no murmurs or gallops.

Respiratory: Clear to auscultation bilaterally.

Abdomen: Soft, non-tender, no organomegaly.

4. ASSESSMENT

Stage 1 Hypertension (ICD-10: I10): Newly identified.

Contributing Factors: Family history, high-sodium diet, sedentary lifestyle, and work-related stress.

5. TREATMENT PLAN

Lifestyle Modifications:

Dietary: Transition to a low-sodium (DASH) diet immediately.

Activity: Target 150 minutes of moderate aerobic exercise per week.

Stress Management: Recommended mindfulness or yoga.

Diagnostic Orders:

Complete Blood Count (CBC)

Comprehensive Metabolic Panel (CMP)

Lipid Profile

Urinalysis

Follow-Up:

Return in 4 weeks for a blood pressure re-check and lab review.

Pharmacologic therapy will be considered if readings remain consistently elevated above 130/80 mmHg.

6. PROVIDER SIGNATURE

Electronically Signed By: Dr. Arthur Sterling, MD Internal Medicine Specialist
Reg No: 88721-B